



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : ناهد ثابت تمام طه
 المحافظة : الجيزة
 الحي / المدينة : العمرانية
 رقم البطاقة : 27109041401048 رقم التليفون : 01112265182
 العمر (تاريخ الميلاد) : 04-09-1971 المهنة : ربه منزل
 العنوان : 3 ش جمال عبدالناصر
 تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
 الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
 نفقات العلاج في حالة ثبوت غير ذلك
 الإخصائي الاجتماعي : _____
 التوقيع: (_____)
 المقر بما فيه
 التوقيع: (_____)

التشخيص الإكلينيكي :

نتائج الأبحاث : (مرفق)

التشخيص النهائي :

خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6688] قسم العلاج :

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
 الاسم : د/ندى حسن
 الاسم : د/نرمين رمضان
 الوظيفة : _____
 الوظيفة : _____
 الوظيفة : _____
 التوقيع : _____
 التوقيع : _____
 التوقيع : _____
 تعتمد صحة الأبحاث والمرفقة
 رئيس اللجنة ومدير
 المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
 بالمستشفى قبل الصرف.

أعضاء اللجنة

د.ا. _____ د.ا. _____ د.ا. _____ د. _____ د. _____

قرار اللجنة

التشخيص النهائي : _____
 خطة العلاج : _____
 المستشفى : _____
 جهة العلاج : _____
 ملاحظات : _____
 قسم : _____
 نفقات : _____

ختم
 شعار الجمهورية

مدير عام
 مديرية الشؤون الصحية

رئيس
 المجلس الطبي المتخصص

Name: ناهد ثابت تمام طه
National ID: 27109041401048



تقرير طبي

المريض

تحية طبية وبعد،،

بشأن تقرير طبي للمريض

إهداء للطلب المقدم من

رقم طبي:

بقسم:

تنتهز هذه بان توضح لهما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الطول :
--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

A patient of ovarian cancer for Supportive and palliative Treatment

الوقت :

mm-dd-yy

التاريخ :

05-07-2026

الطبيب المعالج :
د. الشيماء لاشين

Date Of Submission:

Date Of MDT:



2019
9-10

General MDT form

Patient Name:	نور ثروت محمد	
Sex:	Female	
ID:	13855	
Family History:		
Surgical History:		
Menopausal Status:	POST	
Referring Physician:	MDT	
MDT Name:	Thoracic	▼
Age:	55 Y	
Special Habits:	NAD	27109041401048
Medical ID:	13855	
Comorbidity:	OLD DVT , ON REVAROSPIRE 10MG	
Telephone:	01112265102	
Marital Status:	MARRIED, POST HYSTERECTOMY & LT SO	3/2026
Reason Of MDT:		

Medical Report

Diagnosis:		
Pathology:	TAM AND LT SO WAS FREE OF MALIGNANCY + PERITONIAL FLUID> MARKED MESOTHELIAL IRRITATION.	
IHC:	CA 125 71, AND AFTER 2 MONTHS 55 CEA , CA 19-9 NORMAL	
T:		N:
M:		
History:		
Radiology Investigation:		
Recommendations :	LARGE RT ADIENEAL MASS FOR EXPLORATION AND RT SALPINGO OOPHRECTOMY	
MDT DECISION:	FOR EXPLORATION AND RT SALPINGO OOPHRECTOMY after patient counselling that the lesion is mostly non-malignant	

Signature	Medical Oncology	Surgery	Radiology	Pathology	Nutrition	Psychiatry
	د. خالد بن الحارث	د. محمد العربي				

جمهورية مصر العربية
بطاقة الهوية الشخصية

لأهـ
ثابت تمام طه
٣ ش جمال عبدالناصر
العمرانية - الجيزة

١٩٧١/٠٩/١٤ ٢٧١.٩٠٤١٤.١٠٤٨

JQ9105992



Dr.

Elia Anis Ishak

M. Sc. 2nd D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ناهد ثابت تمام

Analysis ID : 1039316

Age : 54 Y : 6 M

Gender : Female

Conf. Code : 9572

Referred By Prof. Dr. :

Clinical Diagnosis : Ruptured ovarian cyst.

Nature of Specimen : Total abdominal hysterectomy & Left salpingo-oophorectomy and peritoneal fluid.

Receiving Date : 2026-03-05

Patient ID : 762985

PATHOLOGY REPORT

Gross :

Two containers were received designated as:

1-Total abdominal hysterectomy & left salpingo-oophorectomy specimen,
Uterus measuring 8.5x8x6 cm. Endometrial thickness 0.2 cm and there was polypoid tan yellow lesion measured 2.5x2 cm. Myometrial thickness 2.5 cm. There are multiple intramural and subserous well circumscribed masses with whorly firm grayish white cut section, the largest measured 3.5x3x2 cm with focal degenerated area.
Cervix 2.7 cm long, 3 cm in diameter with rubbery grayish white cut section with nabothian cyst.

LT ovary measuring 2.7x2x1.5. cm, left tube measured 5 cm.

2- Peritoneal fluid: 6 cc of red turbid fluid received for cytological examination.

Microscopic :

1-Sections examined from the uterus received revealed endometrial lining showing rounded endometrial glands lined by columnar non secreting epithelial cells with tall nuclei and nuclear pseudostratification. The stroma is formed of spindle cells.

A polypoid mass with thick walled blood vessels and fibrotic core together with rounded and elongated cystically dilated endometrial glands lined by columnar non secreting epithelium with tall dark nuclei and nuclear pseudostratification.

Sections examined from the intramural and subserous masses revealed circumscribed masses formed of interlacing bundles of smooth muscle fibres with no mitotic activity. The largest showed hyaline change. There is evidence of hemorrhage and cholesterol crystals. Sections examined from the cervix revealed cervical tissue covered by mucous secreting columnar endocervical cells. The underlying substantia propria is fibrotic and showed inflammatory reaction. There are tunnel clusters and nabothian cysts.

Sections examined from left ovary revealed cellular stroma.

Sections examined from left tube revealed unremarkable tubes.

No evidence of malignancy in sections examined.

2-Cytological examination of the films and cell block prepared from the centrifugate of the fluid received revealed red blood cells, lymphocytes, polymorphonuclear leucocytes, many mesothelial cells with irritated forms.



عصارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢٨٧.٠٥٢١٧ - ٠١٢١٢٩٤٥١١٥

1/1

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ناهد ثابت تمام

Age : 54 Y : 6 M

Gender : Female

Analysis ID : 1039316

Conf. Code : 9572

Referred By Prof. Dr. :

Clinical Diagnosis : Ruptured ovarian cyst.

Nature of Specimen : Total abdominal hysterectomy & Left salpingo-oophorectomy and
peritoneal fluid.

Receiving Date : 2026-03-05

Patient ID : 762985

PATHOLOGY REPORT

No cytological malignancy in films and cell block examined.

Diagnosis :

- 1-Total abdominal hysterectomy & Left salpingo-oophorectomy, DISORDERED PROLIFERATIVE ENDOMETRIUM, HYPERPLASTIC ENDOMETRIAL POLYP WITH CYSTIC CHANGE, MULTIPLE LEIOMYOMATA THE LARGEST SHOWED HYALINE CHANGE AND EVIDENCE OF HEMORRHAGE, CHRONIC ENDOCERVICITIS, WITH NABOTHIAN CYSTS.
- 2-Peritoneal fluid, Cytology, HAEMORRHAGIC EXUDATIVE REACTION WITH MARKED MESOTHELIAL IRRITATION.

Prof. Dr. Shady Elia Anis



Prof. Dr. Elia Anis Ishak



This report is electronically signed , any abrasion or modification would render it void .

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢٨٧٠٠٥٢١٧ - ٠١٢١٢٩٤٥١١٥

Prof. Dr.

Hany M. Khattab

M.B., B.Ch., M.Sc., M.D. (Pathology)

Professor of Pathology

Faculty of Medicine, Cairo University

Consultant GIT & Hepatopathology

Fellow, Institute Pasteur, Lyon, France

Member of IAP (Arab Division)



الأستاذ الدكتور

هانى محمود خطاب

دكتوراه تحليل الأنسجة والخلايا والأورام

أستاذ الباثولوجى بطب القاهرة

رئيس قسم الباثولوجى سابقا

استشارى أمراض وأورام الجهاز الهضمى والكبد

رئيس معهد باسٲير - ليون فرنسا

عضو الأكاديمية الدولية للباثولوجيا

PATHOLOGY REPORT

Name : **Nahed Thabet**

Date : **May. 2026**

Code : **K-5-188 (Review Slides)**

الاسم : **ناهة ثابت تمام**

التاريخ : **٢٠٢٦/٥/١١**

الكود : **أ.د. / صالح محمود صالح**

Gross:

15 stained slides for revision.

Diagnosis:

HYSTERECTOMY SPECIMEN (REVIEW SLIDES):

- MILD CHRONIC CERVICITIS.
- SIMPLE ENDOMETRIAL HYPERPLASIA WITH HYPERPLASTIC ENDOMETRIAL POLYP.
- FOCAL UTERINE ADENOMYOSIS.
- MULTIPLE BENIGN UTERINE LEIOMYOMATA.
- FEW CORPORA ALBICANTIA OF THE OVARIES.
- THE SMEARS EXAMINED REVEALED MANY RED BLOOD CELLS AND FEW LYMPHOCYTES
- FREE OF ATYPIA OR MALIGNANCY.

الإستاذ الدكتور
هانى محمود خطاب

ه/ك/ب

Prof. Dr. Hany Khattab, M.D

Pat. Name: Nahed Thabet Tammam Pat. ID: 3641595
Procedure Name: Lower Limb Veins Date: 05/09/2026
(One Side)

DUPLEX ASSESSMENT OF THE RIGHT LOWER LIMB VENOUS SYSTEM

- The deep venous system of the right lower extremity; starting from the common femoral vein and down to the popliteal vein and the infrapopliteal veins; all are patent, and totally compressible with no evidence of D.V.T. Color Doppler maps saturated spontaneous color fill-in. On Doppler spectral tracing, they are competent, elicited normal waveform variation with respiration
- The right saphenofemoral junction is patent and competent
- The great and small saphenous veins are patent, totally compressible, and not dilated with no evidence of superficial thrombophlebitis.

Opinion:

No evidence of DVT or Superficial thrombophlebitis.

Dr. Eslam Hamdy, M.Sc

Name: *Nahed Thabet Tammam Taha*

ID: 5169735

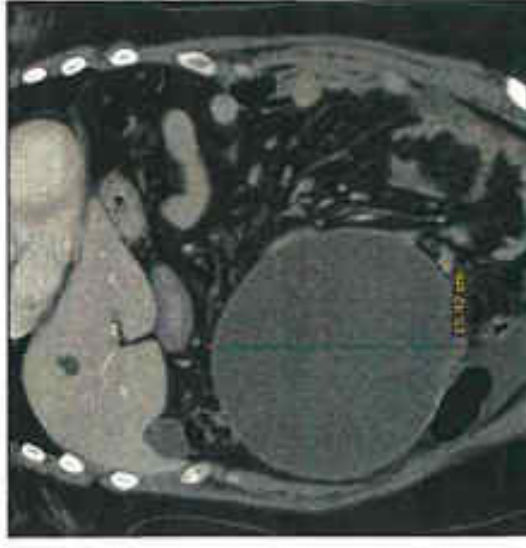
Referred by: *Prof. Saleh Mahmoud Saleh*

Date: 07/05/2026

Multislice post contrast CT scan of the abdomen with special triphasic study and multiplanar reformatted images revealed:

**** Given history of previous intervention.**

- Average sized liver showing:
 - Right hepatic lobe segment VIII and smaller segment VI hypodense focal lesions, the largest at segment VIII measuring about 1.2 x 1.7 cm in axial dimensions with no appreciable post-contrast enhancement.
 - Segment I (caudate lobe) as well as segment IVb focal lesions, showing heterogenous enhancement better appreciated in the arterio-portal phase and being isodense to the hepatic parenchyma in the delayed phase. The largest at segment IVb measuring about 1.7 cm.
- Patent portal vein and its branches.
- Lower anterior abdominal wall subcutaneous scarring with surrounding fat stranding.
- Evidence of hysterectomy with related ill-defined sheets along the lateral aspects of the stump.
- A large cystic lesion is seen along the right aspect of the pelviabdominal cavity, it measures about 12.4 x 16.4 x 15.4 cm in axial and craniocaudal dimensions, respectively.
 - *It shows areas of irregular mural thickening along its posterior wall with related inferior aspect soft tissue density ? ovarian tissue ? soft tissue component.*
 - It is seen displacing the ascending colon along its right posterolateral aspect.



- S C A** Splaying the adjacent small bowel loops along its left and inferior aspects.
- Abutting and stretching the right anterior abdominal wall muscles with overlying contour bulge.
 - Associated surrounding fat stranding.
 - Mild free pelvic fluid with stranding of peritoneal fat planes beneath the right anterior abdominal wall.
 - Normal CT appearance of the pancreas, adrenals, both kidneys (bilateral renal cortical cysts), aorta & IVC.
 - No CT evidence of sizable retro-crural or retro-peritoneal lymph node enlargement.
 - Scanned lung bases showed subsegmental atelectatic changes.

OPINION:

- **Right pelvi-abdominal large cystic lesion as described, possibly adnexal in origin. For correlation with clinical data, interventional details and further assessment.**
- **Right hepatic lobe hypodense non-enhancing focal lesions, likely cysts.**
- **Hepatic segments I and IV focal lesions showing non-specific pattern of enhancement.**
- **Mild pelvic ascites with stranding of peritoneal fat planes beneath the right anterior abdominal wall.**

Dr. Mostafa Kfairy, MD

Dr. Ahmed ElHenry

ID: 5169735



Mega Scan

H A F E

حافظ شريف

للأشعة والتحاليل الطبية

Clinic 10066

Patient Name: Nahed Thabet Tammam	Pat. ID: 80139195
Ref. Doctor: ^{ال} عالية الطبية	Study Date 17/08/2026
Insurance Company:	

Abdomen and pelvis female MRI

FINDINGS:

OPERATIVE BED: Postoperative changes related to hysterectomy are noted. Omental fat standing and mil ascites sis noted at the operative bed.

MASS: Right pelvi-abdominal cystic mass is noted measuring 12 x 14 x 11.5 cm. The lesion shows multiple septations and soft tissue components. The lesion exerts mass effect upon the adjacent structures with peripheral displacement of the surrounding bowel loops. Surrounding fat stranding and minimal collection are noted.

LIVER: Normal size with smooth regular surface. Right hepatic lobe segment VIII cyst is noted measuring 15 mm. Hepatic segment I small lesion measuring 5 mm is noted.

The portal vein and its two divisions appear patent, with preserved vascular flow void. The hepatic biliary system is not dilated.

SPLEEN: Average size. No splenic focal lesion.

PERITONEUM: Anterior abdominal omental nodular thickening is noted.

KIDNEYS: Normal size, parenchymal thickness and signal intensity.

No back pressure changes. Left renal cortical cyst measuring 28 mm is noted.

ADRENAL GLANDS: Normal size bilaterally, with no mass lesion on either side.

PANCREAS: Normal size and signal intensity. No pancreatic mass lesion. The pancreatic duct is not dilated.

LYMPH NODES: No retroperitoneal, mesenteric or pelvic lymphadenopathy.

URINARY BLADDER AND URETHRA: Unremarkable.

BONY PELVIS: Normal bone marrow signal.

IMPRESSION

Postoperative changes related to hysterectomy are noted. Omental fat standing and mil ascites sis noted at the operative bed.

Right pelvi-abdominal cystic mass with multiple septations and soft tissue components.

Correlation with CA 125 and CEA tumor markers is recommended for further evaluation.

Right hepatic lobe small cyst and segment I non-specific small lesion.

Anterior abdominal omental nodular thickening with mild ascites.



19186

Dr. Ahmed El-Gizawy, MD

callcenter@megascancenters.

Bob El Louk Branch :180 El Tahvir St., El Fekky Square

Mohandseen Branch :44 Sylia St., Mohandseen

Giza Branch :10 Murad St., Giza Square

(New)Zayed Branch :Twin Tower, El-Sheikh Zayed,

Tel. : 27943001/ 27944430 / 0128800030

Tel. : 37625577 / 37685588 / 0128800030

Tel. : 35736460 / 35720877 / 0128800030

Tel. : 37939050/ 37939060/ 012040000

Name: Nahed Thabet Tammam

Age : 50 yrs

Date: 03/03/2026

CT SCAN OF THE ABDOMEN AND PELVIS

This is a primary urgent report to be fulfilled later

- Large heterogeneously enhancing soft tissue mass lesion seen at the right side of the pelvis with a small pedicle related to the right adnexal region.
- The lesion measures roughly 12.5 x 17.5 x 15 cm in its AP X TR X CC dimensions respectively. It shows central areas of cystic degeneration.
- The lesion shows clear line of differentiation between it and the uterine fundus.
- Associated moderate pelviabdominal ascites is noted.
- Few enhancing lesions seen in the anterior and posterior uterine walls. Some of the in the anterior wall show cystic changes.

RESSION:

, 22909007

www.phc.org.eg

www.facebook.com/phc.org.eg
info@phc.org.eg

16 شارع السورة - أرض الجولف
مصر الجديدة - القاهرة
تليفون : 2290.9007 - 2290.9008
فاكس : 2290.9008



- Right adnexal soft tissue mass lesion as described mostly of right ovarian origin associated with moderate ascites for histopathological correlation.
- Few uterine lesions mostly fibroids.

Dr. Eman Sameh, MD

Imaging report



كايرو سكين
CAIRO SCAN
Radiology & Labs

Name: Nahed Thabet Tammam	Pat. ID: 3635534
	Date: 29/08/2026

Whole body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- 51 years old patient with hysterectomy and left salpengio - oophorectomy path proven uterine cancer.
- PET/CT requested for assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.
- Hepatic Reference activity~3.7.



Report

No previous PET-CT available for comparison.

Head and Neck:

- No hypermetabolic cervical lymphadenopathy could be detected.
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

- No hypermetabolic axillary or mediastinal lymphadenopathy.
- No FDG avid lung nodules or masses.
- Bilateral lung atelectatic changes
- There are no active pleural abnormalities.
- Physiologic FDG uptake is noted in the heart.



Imaging report

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

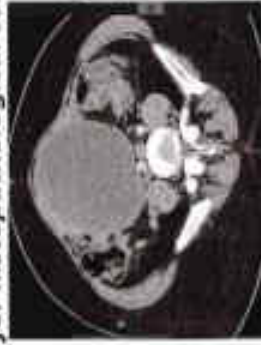
Name: Nahed Thabet Tammam	Pat. ID: 3635534
	Date: 29/08/2026

Abdomen and Pelvis:

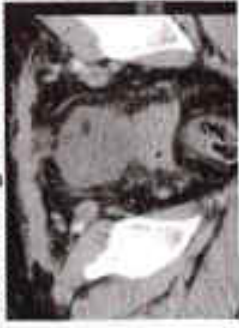
- * Status post hysterectomy and left oophorectomy showing bulky hysterectomy stump with no focal FDG avid lesions detected.



- * Low grade marginally metabolically active FDG avid large right pelvic / adnexal cystic lesion reaching intra-abdominal , abutting anterior abdominal wall and inseparable from adjacent bowel loops, measuring roughly about 12.4x11.9 cm with SUVmax~4.5, for histopathological correlation.



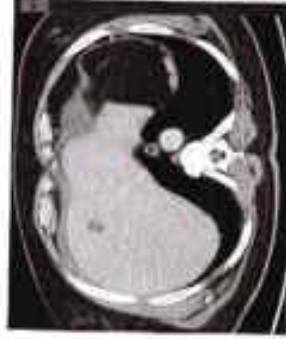
- * Mild abdominopelvic ascites
- * Low grade FDG avid bilateral internal iliac lymph nodes, measuring about 9mm with SUVmax~1.3 for follow up.



- * Physiologic diffuse FDG uptake is seen over average sized liver with homogenous parenchymatous texture. No FDG avid focal lesions detected, metabolically INACTIVE hepatic segment IVa hypodense lesion measuring about 1.6 cm likely of benign nature; hepatic cyst.

Imaging report

Name: Nahed Thabet Tammam	Pat. ID: 3635534
	Date: 29/08/2026



- Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- The pancreas shows normal FDG avidity.
- There are no active adrenal nodules.
- Physiologic FDG excretion is seen in the kidneys and bladder, left renal cortical cyst.
- UB stone.

Musculoskeletal:

- Spondylo-degenerative changes of the spine.
- Normal FDG activity is seen over the axial skeleton.
- No focal active osteoblastic or lytic lesions noted.

Imaging report



Name: Nahed Thabet Tammam	Pat. ID: 3635534
	Date: 29/08/2026

Conclusion:

PET-CT study showing:

- Status post hysterectomy and left oophorectomy showing bulky hysterectomy stump with no focal FDG avid lesions detected.
- Low grade marginally metabolically active FDG avid large right pelvic / adnexal cystic lesion as detailed above for histopathological and tumor markers correlation.
- Mild abdominopelvic ascites
- Low grade FDG avid bilateral internal iliac lymph nodes follow up
- Metabolically INACTIVE hepatic segment IVa hypodense lesion likley of benign nature; hepatic cyst.

Thank you for referral

Dina Thabet , MSc
Nuclear medicine specialist
National Cancer Institute.

Esraa Elkholy , MD
Professor of nuclear medicine
National Cancer Institute

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg

Date Of Submission:

Date Of MDT:



2019
9-10

General MDT form

Patient Name:	نور ثروت محمد	
Sex:	Female	
ID:	13855	
Family History:		
Surgical History:		
Menopausal Status:	POST	
Referring Physician:	MDT	
MDT Name:	Thoracic	▼
Age:	55 Y	
Special Habits:	NAD	27109041401048
Medical ID:	13855	
Comorbidity:	OLD DVT , ON REVAROSPIRE 10MG	
Telephone:	01112265182	
Marital Status:	MARRIED, POST HYSTERECTOMY & LT SO	3/2026
Reason Of MDT:		

Medical Report

Diagnosis:		
Pathology:	TAM AND LT SO WAS FREE OF MALIGNANCY + PERITONIAL FLUID> MARKED MESOTHELIAL IRRITATION.	
IHC:	CA 125 71, AND AFTER 2 MONTHS 55 CEA , CA 19-9 NORMAL	
T:		N:
M:		
History:		
Radiology Investigation:		
Recommendations :	LARGE RT ADIENEAL MASS FOR EXPLORATION AND RT SALPINGO OOPHRECTOMY	
MDT DECISION:	FOR EXPLORATION AND RT SALPINGO OOPHRECTOMY after patient counselling that the lesion is mostly non-malignant	

Signature	Medical Oncology	Surgery	Radiology	Pathology	Nutrition	Psychiatry
	د. خالد بن الحارث	د. محمد العربي				

جمهورية مصر العربية
بطاقة الهوية الشخصية

لأهـ
ثابت تمام طه
٣ ش جمال عبدالناصر
العمرانية - الجيزة

١٩٧١/٠٩/١٤ ٢٧١.٩٠٤١٤.١٠٤٨

JQ9105992

٢٠٢٢/١١ ٢٧١٠٩٠٤١٤٠١٠٤٨
معامية حرة

أنش مسلمة متروجة
حسام عبداللطيف حداد رضوان
البطاقة مارية حتى ٢٠٢٤/١١/١٩





التاريخ ٦ / ٩ / ٢٠٢٣

لا يعتد به كشهادة طبية

جريدة الرياض - ١٠ شارع القدس العربي - أسباطس - الهرم

المشيرة برقم ٤٠٧ جيزة

العيادة المشتركة

الاسم: / / / / /

if pt need any surgery
(surgical operation) start
cleaning & in 1/12h before
5 day pre op, stop 24h
before operation, start some Dose
of op if possible for 5 day

د. محمد بن عبد الله
د. أحمد بن محمد

الإستشارة خلال أسبوع من تاريخ الكشف

العيادة تستقبل حالات الولادة والحوادث ٢٤ ساعة طوال أيام الأسبوع ت: ٢٢٨١٧٩٢٢ - ٢٢٨١٥٥٥٥

Dr.

Elia Anis Ishak

M. Sc. 2nd D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ناهد ثابت تمام

Age : 54 Y : 6 M

Gender : Female

Analysis ID :  1039316

Conf. Code : 9572

Referred By Prof. Dr. :

Clinical Diagnosis : Ruptured ovarian cyst.

Nature of Specimen : Total abdominal hysterectomy & Left salpingo-oophorectomy and peritoneal fluid.

Receiving Date : 2026-03-05

Patient ID : 762985

PATHOLOGY REPORT

Gross :

Two containers were received designated as:

1-Total abdominal hysterectomy & left salpingo-oophorectomy specimen,
Uterus measuring 8.5x8x6 cm. Endometrial thickness 0.2 cm and there was polypoid tan yellow lesion measured 2.5x2 cm. Myometrial thickness 2.5 cm. There are multiple intramural and subserous well circumscribed masses with whorly firm grayish white cut section, the largest measured 3.5x3x2 cm with focal degenerated area.
Cervix 2.7 cm long, 3 cm in diameter with rubbery grayish white cut section with nabothian cyst.

LT ovary measuring 2.7x2x1.5. cm, left tube measured 5 cm.

2- Peritoneal fluid: 6 cc of red turbid fluid received for cytological examination.

Microscopic :

1-Sections examined from the uterus received revealed endometrial lining showing rounded endometrial glands lined by columnar non secreting epithelial cells with tall nuclei and nuclear pseudostratification. The stroma is formed of spindle cells.

A polypoid mass with thick walled blood vessels and fibrotic core together with rounded and elongated cystically dilated endometrial glands lined by columnar non secreting epithelium with tall dark nuclei and nuclear pseudostratification.

Sections examined from the intramural and subserous masses revealed circumscribed masses formed of interlacing bundles of smooth muscle fibres with no mitotic activity. The largest showed hyaline change. There is evidence of hemorrhage and cholesterol crystals. Sections examined from the cervix revealed cervical tissue covered by mucous secreting columnar endocervical cells. The underlying substantia propria is fibrotic and showed inflammatory reaction. There are tunnel clusters and nabothian cysts.

Sections examined from left ovary revealed cellular stroma.

Sections examined from left tube revealed unremarkable tubes.

No evidence of malignancy in sections examined.

2-Cytological examination of the films and cell block prepared from the centrifugate of the fluid received revealed red blood cells, lymphocytes, polymorphonuclear leucocytes, many mesothelial cells with irritated forms.



Dr.
Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ناهد ثابت تمام

Analysis ID : 1039316

Age : 54 Y : 6 M

Gender : Female

Conf. Code : 9572

Referred By Prof. Dr. :

Clinical Diagnosis : Ruptured ovarian cyst.

Nature of Specimen : Total abdominal hysterectomy & Left salpingo-oophorectomy and
peritoneal fluid.

Receiving Date : 2026-03-05

Patient ID : 762985

PATHOLOGY REPORT

No cytological malignancy in films and cell block examined.

Diagnosis :

- 1-Total abdominal hysterectomy & Left salpingo-oophorectomy, DISORDERED PROLIFERATIVE ENDOMETRIUM, HYPERPLASTIC ENDOMETRIAL POLYP WITH CYSTIC CHANGE, MULTIPLE LEIOMYOMATA THE LARGEST SHOWED HYALINE CHANGE AND EVIDENCE OF HEMORRHAGE, CHRONIC ENDOCERVICITIS, WITH NABOTHIAN CYSTS.
- 2-Peritoneal fluid, Cytology, HAEMORRHAGIC EXUDATIVE REACTION WITH MARKED MESOTHELIAL IRRITATION.

Prof. Dr. Shady Elia Anis



Prof. Dr. Elia Anis Ishak



This report is electronically signed , any abrasion or modification would render it void .

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢٨٧٠٠٥٢١٧ - ٠١٢١٢٩٤٥١١٥

Prof. Dr.

Hany M. Khattab

M.B., B.Ch., M.Sc., M.D. (Pathology)

Professor of Pathology

Faculty of Medicine, Cairo University

Consultant GIT & Hepatopathology

Fellow, Institute Pasteur, Lyon, France

Member of IAP (Arab Division)



الأستاذ الدكتور

هانى محمود خطاب

دكتوراه تحليل الأنسجة والخلايا والأورام

أستاذ الباثولوجى بطب القاهرة

رئيس قسم الباثولوجى سابقا

استشارى أمراض وأورام الجهاز الهضمى والكبد

رئيس معهد باسستير - ليون فرنسا

عضو الأكاديمية الدولية للباثولوجيا

PATHOLOGY REPORT

Name : **Nahed Thabet**

Date : **May. 2026**

Code : **K-5-188 (Review Slides)**

الاسم : **ناهة ثابت تمام**

التاريخ : **٢٠٢٦/٥/١١**

الكود : **أ.د. / صالح محمود صالح**

Gross:

15 stained slides for revision.

Diagnosis:

HYSTERECTOMY SPECIMEN (REVIEW SLIDES):

- MILD CHRONIC CERVICITIS.
- SIMPLE ENDOMETRIAL HYPERPLASIA WITH HYPERPLASTIC ENDOMETRIAL POLYP.
- FOCAL UTERINE ADENOMYOSIS.
- MULTIPLE BENIGN UTERINE LEIOMYOMATA.
- FEW CORPORA ALBICANTIA OF THE OVARIES.
- THE SMEARS EXAMINED REVEALED MANY RED BLOOD CELLS AND FEW LYMPHOCYTES
- FREE OF ATYPIA OR MALIGNANCY.

الأستاذ الدكتور
هانى محمود خطاب

Hany M. Khattab

Prof. Dr. Hany Khattab, M.D

Pat. Name: Nahed Thabet Tammam Pat. ID: 3641595
Procedure Name: Lower Limb Veins Date: 05/09/2026
(One Side)

DUPLEX ASSESSMENT OF THE RIGHT LOWER LIMB VENOUS SYSTEM

- The deep venous system of the right lower extremity; starting from the common femoral vein and down to the popliteal vein and the infrapopliteal veins; all are patent, and totally compressible with no evidence of D.V.T. Color Doppler maps saturated spontaneous color fill-in. On Doppler spectral tracing, they are competent, elicited normal waveform variation with respiration
- The right saphenofemoral junction is patent and competent
- The great and small saphenous veins are patent, totally compressible, and not dilated with no evidence of superficial thrombophlebitis.

Opinion:

No evidence of DVT or Superficial thrombophlebitis.

Dr. Eslam Hamdy, M.Sc

PATIENT NAME		Registered	29-04-2026 21:41:27	Collected	29-04-2026 21:42:09
تأه ثابت تمام طة		Authenticated	30-04-2026 15:07:35	Printed	01-05-2026 14:52:18
Visit Number	Age	Gender	Referred By		
30926503612	50 Year	Female	صالح محمود صالح		
				Client ID	1618

Test Name Result Unit Reference Range Previous Result

Tumor Markers

Carcinoembryonic Antigen (CEA)

1.45

ng/mL

Non smoker: up to 3.0
Smoker: up to 5.0

Assay Method: CLIA/ECLIA

Results determined by using different methods may not be comparable.

CA 19-9

32

U/mL

0 - 34

Assay Method: ECLIA

Results determined by using different methods may not be comparable.

CA 125

71.00

U/mL

0 - 35

Follow up is recommended

Assay Method: CLIA/ECLIA

Results determined by using different methods may not be comparable.



A. Norka

Dr. ABD El-Aziz El-Norka

Professor of Medical Bio-Chemistry,
Faculty of Medicine, Al Azhar University

1-1

Name: *Nahed Thabet Tammam Taha*

ID: 5169735

Referred by: *Prof. Saleh Mahmoud Saleh*

Date: 07/05/2026

Multislice post contrast CT scan of the abdomen with special triphasic study and multiplanar reformatted images revealed:

**** Given history of previous intervention.**

- Average sized liver showing:
 - Right hepatic lobe segment VIII and smaller segment VI hypodense focal lesions, the largest at segment VIII measuring about 1.2 x 1.7 cm in axial dimensions with no appreciable post-contrast enhancement.
 - Segment I (caudate lobe) as well as segment IVb focal lesions, showing heterogenous enhancement better appreciated in the arterio-portal phase and being isodense to the hepatic parenchyma in the delayed phase. The largest at segment IVb measuring about 1.7 cm.
- Patent portal vein and its branches.
- Lower anterior abdominal wall subcutaneous scarring with surrounding fat stranding.
- Evidence of hysterectomy with related ill-defined sheets along the lateral aspects of the stump.
- A large cystic lesion is seen along the right aspect of the pelviabdominal cavity, it measures about 12.4 x 16.4 x 15.4 cm in axial and craniocaudal dimensions, respectively.
 - *It shows areas of irregular mural thickening along its posterior wall with related inferior aspect soft tissue density ? ovarian tissue ? soft tissue component.*
 - It is seen displacing the ascending colon along its right posterolateral aspect.



- S C A** Splaying the adjacent small bowel loops along its left and inferior aspects.
- Abutting and stretching the right anterior abdominal wall muscles with overlying contour bulge.
 - Associated surrounding fat stranding.
 - Mild free pelvic fluid with stranding of peritoneal fat planes beneath the right anterior abdominal wall.
 - Normal CT appearance of the pancreas, adrenals, both kidneys (bilateral renal cortical cysts), aorta & IVC.
 - No CT evidence of sizable retro-crural or retro-peritoneal lymph node enlargement.
 - Scanned lung bases showed subsegmental atelectatic changes.

OPINION:

- **Right pelvi-abdominal large cystic lesion as described, possibly adnexal in origin. For correlation with clinical data, interventional details and further assessment.**
- **Right hepatic lobe hypodense non-enhancing focal lesions, likely cysts.**
- **Hepatic segments I and IV focal lesions showing non-specific pattern of enhancement.**
- **Mild pelvic ascites with stranding of peritoneal fat planes beneath the right anterior abdominal wall.**

Dr. Mostafa Kfairy, MD

Dr. Ahmed ElHenry

ID: 5169735



Mega Scan

H A F E

حافظ شريف

للأشعة والتحاليل الطبية

Clinic 10066

Patient Name: Nahed Thabet Tammam	Pat. ID: 80139195
Ref. Doctor: ^{ال} عالية الطبية	Study Date 17/08/2026
Insurance Company:	

Abdomen and pelvis female MRI

FINDINGS:

OPERATIVE BED: Postoperative changes related to hysterectomy are noted. Omental fat standing and mil ascites sis noted at the operative bed.

MASS: Right pelvi-abdominal cystic mass is noted measuring 12 x 14 x 11.5 cm. The lesion shows multiple septations and soft tissue components. The lesion exerts mass effect upon the adjacent structures with peripheral displacement of the surrounding bowel loops. Surrounding fat stranding and minimal collection are noted.

LIVER: Normal size with smooth regular surface. Right hepatic lobe segment VIII cyst is noted measuring 15 mm. Hepatic segment I small lesion measuring 5 mm is noted.

The portal vein and its two divisions appear patent, with preserved vascular flow void. The hepatic biliary system is not dilated.

SPLEEN: Average size. No splenic focal lesion.

PERITONEUM: Anterior abdominal omental nodular thickening is noted.

KIDNEYS: Normal size, parenchymal thickness and signal intensity.

No back pressure changes. Left renal cortical cyst measuring 28 mm is noted.

ADRENAL GLANDS: Normal size bilaterally, with no mass lesion on either side.

PANCREAS: Normal size and signal intensity. No pancreatic mass lesion. The pancreatic duct is not dilated.

LYMPH NODES: No retroperitoneal, mesenteric or pelvic lymphadenopathy.

URINARY BLADDER AND URETHRA: Unremarkable.

BONY PELVIS: Normal bone marrow signal.

IMPRESSION

Postoperative changes related to hysterectomy are noted. Omental fat standing and mil ascites sis noted at the operative bed.

Right pelvi-abdominal cystic mass with multiple septations and soft tissue components.

Correlation with CA 125 and CEA tumor markers is recommended for further evaluation.

Right hepatic lobe small cyst and segment I non-specific small lesion.

Anterior abdominal omental nodular thickening with mild ascites.



Dr. Ahmed El-Gizawy, MD

19186

callcenter@megascancenters.

Bob El Louk Branch :180 El Tahvir St., El Fekky Square

Mohandseen Branch :44 Sylia St., Mohandseen

Giza Branch :10 Murad St., Giza Square

(New)Zayed Branch :Twin Tower, El-Sheikh Zayed,

Tel. : 27943001/ 27944430 / 0128800030

Tel. : 37625577 / 37685588 / 0128800030

Tel. : 35736460 / 35720877 / 0128800030

Tel. : 37939050/ 37939060/ 012040000

Name: Nahed Thabet Tammam

Age : 50 yrs

Date: 03/03/2026

CT SCAN OF THE ABDOMEN AND PELVIS

This is a primary urgent report to be fulfilled later

- Large heterogeneously enhancing soft tissue mass lesion seen at the right side of the pelvis with a small pedicle related to the right adnexal region.
- The lesion measures roughly 12.5 x 17.5 x 15 cm in its AP X TR X CC dimensions respectively. It shows central areas of cystic degeneration.
- The lesion shows clear line of differentiation between it and the uterine fundus.
- Associated moderate pelviabdominal ascites is noted.
- Few enhancing lesions seen in the anterior and posterior uterine walls. Some of the in the anterior wall show cystic changes.

RESSION:

, 22909007

www.phc.org.eg

www.facebook.com/phc.org.eg
Info@phc.org.eg

16 شارع السورة - أرض الجولف
مصر الجديدة - القاهرة
تليفون : ٢٢٩٠٩٠٠٨ - (٠٢٠٢) ٢٢٩٠٩٠٠٧
فاكس : ٢٢٩٠١٠٠٨ - (٠٢٠٢)



- Right adnexal soft tissue mass lesion as described mostly of right ovarian origin associated with moderate ascites for histopathological correlation.
- Few uterine lesions mostly fibroids.

Dr. Eman Sameh, MD

Imaging report

Name: Nahed Thabet Tammam	Pat. ID: 3635534
	Date: 29/08/2026

Whole body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- 51 years old patient with hysterectomy and left salpengio - oophorectomy path proven uterine cancer.
- PET/CT requested for assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.
- Hepatic Reference activity~3.7.



Report

No previous PET-CT available for comparison.

Head and Neck:

- No hypermetabolic cervical lymphadenopathy could be detected.
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

- No hypermetabolic axillary or mediastinal lymphadenopathy.
- No FDG avid lung nodules or masses.
- Bilateral lung atelectatic changes
- There are no active pleural abnormalities.
- Physiologic FDG uptake is noted in the heart.



Imaging report

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Dr. Mohamed Elsayed

Name: Nahed Thabet Tammam	Pat. ID: 3635534
	Date: 29/08/2026

Abdomen and Pelvis:

- * Status post hysterectomy and left oophorectomy showing bulky hysterectomy stump with no focal FDG avid lesions detected.



- * Low grade marginally metabolically active FDG avid large right pelvic / adnexal cystic lesion reaching intra-abdominal , abutting anterior abdominal wall and inseparable from adjacent bowel loops, measuring roughly about 12.4x11.9 cm with SUVmax~4.5, for histopathological correlation.



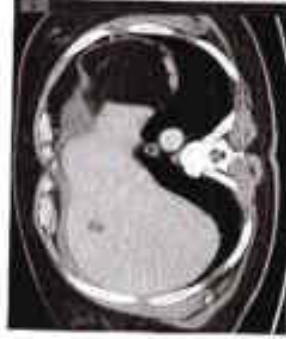
- * Mild abdominopelvic ascites
- * Low grade FDG avid bilateral internal iliac lymph nodes, measuring about 9mm with SUVmax~1.3 for follow up.



- * Physiologic diffuse FDG uptake is seen over average sized liver with homogenous parenchymatous texture. No FDG avid focal lesions detected, metabolically INACTIVE hepatic segment IVa hypodense lesion measuring about 1.6 cm likely of benign nature; hepatic cyst.

Imaging report

Name: Nahed Thabet Tammam	Pat. ID: 3635534
	Date: 29/08/2026



- Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- The pancreas shows normal FDG avidity.
- There are no active adrenal nodules.
- Physiologic FDG excretion is seen in the kidneys and bladder, left renal cortical cyst.
- UB stone.

Musculoskeletal:

- Spondylo-degenerative changes of the spine.
- Normal FDG activity is seen over the axial skeleton.
- No focal active osteoblastic or lytic lesions noted.

Imaging report



Name: Nahed Thabet Tammam	Pat. ID: 3635534
	Date: 29/08/2026

Conclusion:

PET-CT study showing:

- Status post hysterectomy and left oophorectomy showing bulky hysterectomy stump with no focal FDG avid lesions detected.
- Low grade marginally metabolically active FDG avid large right pelvic / adnexal cystic lesion as detailed above for histopathological and tumor markers correlation.
- Mild abdominopelvic ascites
- Low grade FDG avid bilateral internal iliac lymph nodes follow up
- Metabolically INACTIVE hepatic segment IVa hypodense lesion likley of benign nature; hepatic cyst.

Thank you for referral

Dina Thabet , MSc
Nuclear medicine specialist
National Cancer Institute.

Esraa Elkholy , MD
Professor of nuclear medicine
National Cancer Institute

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg



اسم الفرع اليوم 3
مدير الفرع د. أحمد عبد الصبور
مدير الوحدة د. عبد العزيز التقي
طبيب الوحدة د. علا السيسى
د. إيهاب إبراهيم

رئيس مجلس الإدارة
د. مؤمنة كامل

مدير المعامل
د. هبة الشربيني

PATIENT NAME ناهة ثابت تمام طه		Registered	11-08-2026 14:37:34	Collected	11-08-2026 14:39:25
Visit Number 30926506769		Authenticated	11-08-2026 19:07:41	Printed	12-08-2026 18:04:03
Age 51 Year	Gender Female	Referred By محمد النخسي Prof:	Client ID 1618		

Test Name	Result	Unit	Reference Range	Previous Result
Tumor Markers				

Cardinoembryonic Antigen (CEA) 1.86 ng/mL Non smoker; up to 3.0
Smoker; up to 5.0 1.45

Assay Method: CLIA/ECLIA
Results determined by using different methods may not be comparable.

CA 19-9 ↑ 37 U/mL 0 - 34 32

Assay Method: ECLIA
Results determined by using different methods may not be comparable.

CA 125 ↑ 55.40 U/mL 0 - 35 71.00

Follow up is recommended

Assay Method: CLIA/ECLIA
Results determined by using different methods may not be comparable.

Serum LDH 220 U/L 120 - 246



A. Nohaly

Dr.ABD El-Aziz El-Nokaly
Professor of Medical Bio-Chemistry,
Faculty of Medicine, Al Azhar University 1-1

Dr.
Elia Anis Ishak

M. Sc. MD Pathology , Prof. of Pathology , Cairo University
Fellow , Armed Forces Institute of Pathology , USA
Member , United States Canadian Academy of Pathology

Dr.
Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ناهد ثابت تمام

Age : 54 Y : 6 M

Gender : Female

Analysis ID : 1039316

Conf. Code : 9572

Referred By Prof. Dr. :

Clinical Diagnosis : Ruptured ovarian cyst.

Nature of Specimen : Total abdominal hysterectomy & Left salpingo-oophorectomy and
peritoneal fluid.

Receiving Date : 2026-03-05

Patient ID : 762985

PATHOLOGY REPORT

No cytological malignancy in films and cell block examined.

Diagnosis :

1-Total abdominal hysterectomy & Left salpingo-oophorectomy, DISORDERED
PROLIFERATIVE ENDOMETRIUM, HYPERPLASTIC ENDOMETRIAL POLYP WITH
CYSTIC CHANGE, MULTIPLE LEIOMYOMATA THE LARGEST SHOWED HYALINE
CHANGE AND EVIDENCE OF HEMORRHAGE, CHRONIC ENDOCERVICITIS, WITH
NABOTHIAN CYSTS.
2-Peritoneal fluid, Cytology, HAEMORRHAGIC EXUDATIVE REACTION WITH MARKED
MESOTHELIAL IRRITATION.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia anis



This report is electronically signed , any abrasion or modification would render it void .

عصارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تيلفون : ٠١٢٨٧.٠٥٢١٧ - ٠١٢١٢٩٤٥١١٥